BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm completion.

Check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Blood / organ function	full blood count (FBC), renal, liver, glucose	Check before cycle	FBC / renal / liver / glucose each cycle	Confirm Day 8/15 count decisions	Review continuing abnormalities
Immune / thyroid	thyroid function tests (TFT), cortisol, hepatitis B virus (HBV) / hepatitis C virus (HCV)	TFT Cycle 1 Day 1	TFT every second cycle	Continue when chemotherapy stops	Immune symptoms may occur later

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00688 v2a · official regimen · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Atezolizumab + nab-paclitaxel: treatment and checks

Previously untreated unresectable locally advanced or metastatic triple-negative breast cancer with eligible PD-L1 expression

Atezolizumab 840 mg is given on Days 1 and 15, before nab-paclitaxel 100 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Nab-paclitaxel is planned for up to six cycles; extension needs consultant review. Atezolizumab may continue after chemotherapy stops.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Treatment plan NCCP 00688	SP142 IC≥1%; no prior advanced chemotherapy	D1 / D15 both medicines	D8 nab-paclitaxel only	Nab up to 6 cycles; atezolizumab may continue	Immune / disease review
Blood / organ function NCCP 00688	FBC, renal, liver, glucose	Check before cycle	FBC / renal / liver / glucose each cycle	Confirm Day 8/15 count decisions	Review continuing abnormalities
Immune / thyroid NCCP 00688	TFT, cortisol, HBV / HCV	TFT Cycle 1 Day 1	TFT every second cycle	Continue when chemotherapy stops	Immune symptoms may occur later

Care / check	Before starting	First treatment	During treatment	Change / completion	Follow-up
Cancer review METASTATIC · MARKERS	Confirm setting	Agree review plan	Response / symptom review	Next treatment / completion	Agreed surveillance

What each check is for

1 Before treatment

Before the first dose

Previously untreated unresectable locally advanced or metastatic triple-negative breast cancer with eligible PD-L1 expression. Baseline FBC, kidney/liver function, glucose, thyroid function, hepatitis B/C testing and morning cortisol. FBC, kidney/liver tests and glucose before each cycle; check the count results needed for Day 8 and Day 15 decisions.

Ask / **act:** Confirm eligibility, test results and medicine list.

Sources: NCCP 00688

2 Other checks

At the specified review points

Thyroid function at Cycle 1 Day 1 and every second cycle. Review immune symptoms throughout and after treatment. Cardiac imaging is based on history/prior anthracyclines or clinical concern; there is no routine HER2-style echo schedule.

Ask / **act:** Confirm who arranges and reviews each check.

Sources: NCCP 00688

3 Support and practical plan

At each treatment or phase change

Separate combination, Day 8 chemotherapy-only and immunotherapy-only support; maintain immune and blood-count monitoring.

Ask / **act:** Choose the matching support plan; confirm active medicines.

Sources: NCCP 00688

4 Response and follow-up

Agreed review points

Agree the scan type and review interval for the known disease sites. Review response, symptoms and toxicity together; new or worsening symptoms may need an earlier assessment. Keep the agreed follow-up and report persistent or new symptoms between visits.

Ask / **act:** Review symptoms, results and patient priorities.

Sources: METASTATIC · MARKERS

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.19